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Back Care Office

Spinal Care - Long Board Use

To be used on patients who have suspected neck and back injury. Check the patient frequently, improper use could result in death or serious injury.

- **Minimum of five people needed**
- **Check area is safe to enter and clear**
- **Patient to be assessed by a doctor**
- **Check the equipment (board, stitching, straps, buckles and ring closures)**
- **Remove the straps, prepare the receiving surface**
- **Safe working load: 159kg**

At all times the patient's head MUST be controlled and held (both sides of the head). Staff member controlling the head to lead the move.

Log roll patient using three hands on top, three hands under method.
Insert the board under the patient by tipping at an angle. Support the board to the floor. Guide the patient onto the remainder of the board.

Affix the strap using a figure 8 to the patient’s feet, loosely tighten. Affix strap to hip area, loosely tighten. Affix the strap over shoulder and secure the arms, loosely tighten.

Patients head is held by the second carer (chin and top of head). First carer gives command and releases hold of head. Two wedges are positioned against the patient's cheeks.
Forehead strap is secured (both sides at the same time). Chin strap is secured.

All body straps are tightened. You should be able to place four fingers with the hand flat between the patient's body and the strap.

Patient's head is released by second carer.
Patient is lifted from floor.
Receiving surface is positioned under the board.
Board is lowered onto the surface.

Protocol states that a patient should not remain on the spinal board for longer than 20 minutes.